

ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #03

BEGINNER STEROID CYCLE

Complete First-Cycle Guide with PCT & Blood Work Protocol

12 Weeks · 6 Chapters · Beginner Level · Safety First

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DISCLAIMER

This guide is for educational purposes only and does not constitute medical or professional advice. Always consult a qualified healthcare provider before starting any supplementation, pharmaceutical, or training protocol. All information is provided for harm-reduction and educational purposes. Individual results vary.

CHAPTER 1 — IS A STEROID CYCLE RIGHT FOR YOU?

"The most dangerous cycle is the one entered without knowledge. Education is your best protection."

Before beginning any anabolic cycle, it is essential to complete a honest self-assessment. Anabolic steroids carry real risks that are amplified by insufficient natural training base, poor nutrition habits, inadequate sleep, and absence of medical monitoring. This guide is designed for harm reduction — to ensure that if someone is going to use, they do so with maximum safety.

Readiness Checklist

Criterion	Minimum Requirement	Why It Matters
Training age	3+ years consistent resistance training	Maximise receptors, base strength
Age	23+ years (25+ recommended)	HPTA still developing before 23
Blood work	Baseline panel completed	Know your starting point
Body fat	<20% male, <28% female	High BF = high aromatisation risk
Nutrition knowledge	Macro tracking for 6+ months	Compounds amplify habits — good and bad
Support person	Trusted person aware of cycle	Medical emergency preparedness

■ ***If you cannot consistently gain muscle naturally, adding anabolic compounds will not solve the problem. Fix nutrition and training first.***

CHAPTER 2 — FIRST CYCLE: TESTOSTERONE ONLY

The universally recommended first cycle is Testosterone Enanthate or Testosterone Cypionate, 400–500mg per week, for 12 weeks. This is not conservative advice — it is optimal advice. Testosterone is the foundation of all anabolic protocols and introducing multiple compounds simultaneously makes it impossible to identify the source of any side effect.

Week	Testosterone E	Aromasin/Arimidex	HCG	Notes
1–12	250mg Mon + 250mg Thu	Only if gyno symptoms	250IU 2×/wk (optional)	Assess response Week 3
13–14	None	Continue if needed	Discontinue	Clearance period
15–18 (PCT)	None	Taper off	None	Clomid + Nolvadex

■ ***Do NOT take an AI (aromatase inhibitor) pre-emptively. Only introduce one if you develop gynaecological symptoms (itchy, tender nipples). Crashing estrogen is worse than elevated estrogen.***

- Inject with a 23-gauge 1.5" needle into the gluteus medius or ventral gluteus
- Rotate injection sites every time — build a 4-site rotation minimum
- Aspirate optional but a good habit for new users
- Warm the oil to body temperature before injection (hold in hands 2 minutes)
- Draw with an 18G needle, swap to 23G for injection — keep everything sterile

CHAPTER 3 — TRAINING FOR BEGINNERS ON CYCLE

On-cycle beginner training should be higher volume than natural training, but not recklessly so. The biggest mistake beginners make is insufficient volume — muscles on cycle can tolerate and require more stimulus. The second mistake is insufficient recovery — you are not recovering faster on cycle unless you are also sleeping and eating correctly.

Day	Muscle Group	Sets	Rep Range	Rest
Monday	Chest + Triceps	16–18	8–12	90 sec
Tuesday	Back + Biceps	16–18	8–12	90 sec
Wednesday	Legs + Calves	18–20	8–15	2 min
Thursday	Shoulders + Traps	14–16	10–15	90 sec
Friday	Arms + Forearms	14–16	10–15	75 sec
Saturday	Weak points or cardio	12–14	Mixed	60–90 sec
Sunday	Rest + mobility	—	—	—

CHAPTER 4 — NUTRITION FOR MAXIMUM GAINS

"Your compounds are multipliers, not replacements. Multiply poor nutrition and you get amplified poor results."

Meal	Timing	Composition	Approx Kcal
Meal 1	7:00 AM	6 whole eggs + 2 roti + 1 banana	650
Meal 2	10:30 AM	200g chicken + 1 cup rice + salad	550
Pre-workout	1:00 PM	50g oats + 1 scoop whey + 1 apple	450
Post-workout	4:00 PM	2 scoops whey + 50g dextrose	380
Meal 5	7:00 PM	200g paneer/chicken + 2 roti + veg	600
Night meal	10:00 PM	250ml full fat milk + 30g casein	320

Total: ~2950 kcal — adjust based on actual bodyweight and scale movement

CHAPTER 5 — ON-CYCLE SUPPORT & BLOOD WORK

Supplement	Dose	Purpose	Timing
Liv52 / TUDCA	500mg TUDCA or 2 tabs Liv52	Liver protection	With meals
Omega-3	4g EPA/DHA combined	Lipid profile protection	With meals
CoQ10	200mg	Cardiovascular support	Morning
Vitamin D3	5000 IU	Hormonal support, immunity	Morning with fat
Zinc	25mg	Testosterone co-factor	Before bed
NAC	600mg twice daily	Liver + antioxidant	Morning + evening
Hawthorn Berry	500mg	Blood pressure management	Morning

■ **Blood work protocol: Pre-cycle baseline → Week 6 check → 4 weeks post-PCT. This 3-point system catches problems early and confirms recovery.**

CHAPTER 6 — POST CYCLE THERAPY (PCT)

PCT begins 14–18 days after the last testosterone injection (for Enanthate/Cypionate). Do not start PCT earlier — residual testosterone will suppress endogenous recovery.

PCT Week	Clomid	Nolvadex (Tamoxifen)	Notes
Week 1	50mg/day	40mg/day	Expect lethargy, low libido — normal
Week 2	50mg/day	40mg/day	Mood fluctuation peaks this week
Week 3	25mg/day	20mg/day	Energy begins returning
Week 4	25mg/day	20mg/day	Final week — bloods 4 weeks after

- Maintain training intensity and volume through PCT — muscle loss is hormonal + training deficit combined
- Caloric surplus or maintenance through PCT — now is not the time to cut
- Avoid alcohol completely during PCT — liver is processing PCT drugs AND recovering
- Zinc, Vitamin D, ashwagandha all support natural testosterone recovery
- If bloods show testosterone below 300ng/dL at 8 weeks post-PCT, see an endocrinologist

"PCT is not the end of the cycle — it is the most important part of it. Skip it and you risk permanent HPTA suppression."